

# View Letter

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| Date:    | 21 Sep 2026                                              |
| To:      | "Jun Yang" yjun201602@163.com                            |
| From:    | "International Journal of Surgery" ijs@wolterskluwer.com |
| Subject: | Your Submission IJS-D-26-02226                           |

21 Sep 2026

RE: IJS-D-26-02226, entitled "Diagnosis and treatment of wandering spleen in children: A novel splenopexy technique"

Dear Dr. Yang,

The editorial and peer review of your manuscript have now been completed. I regret to inform you that International Journal of Surgery is unable to publish your manuscript, however we believe your work may be more suitable in another of our journals.

The reviewers' comments are included at the bottom of this letter. Please note that the comments are for your information only and do not constitute a request for revisions to International Journal of Surgery.

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#### Reviewer Comments:

Reviewer #1: Our research colleagues have produced an interesting study on the mobile spleen. They must be working at a high-volume center, as they managed to "collect" four cases—a small series—of mobile spleens. The abstract and introduction are good, but we have nothing to add except that sometimes these malformations, which can be classified as malformations, can be accompanied by ptotic kidneys, horseshoe kidneys, cardiac malformations, or joint syndromes. The case descriptions are well-written, and clearly, in cases where splenic infarction occurs, the only option left is splenectomy. Our expert colleagues are well aware that the tail of the pancreas can be involved in splenic pathology, and in cases of splenectomy, decapitation of the Wirsung at the end of the pancreatic tail can result. Fixation of the spleen at both poles is excellent to avoid vessel loops and splenic infarction. Good English, good bibliography, and the images perfectly complement the paper.

Reviewer #2: Thank you for describing your technique and looking forward for the upcoming study with evaluation of bigger number of cases

Reviewer #3: Major Methodological & Structural Concerns  
**Overstated Efficacy of the "Novel" Technique:** The core premise of the paper highlights a "reinforced suture fixation technique for the superior and inferior splenic poles". However, the clinical data reveals that this novel technique was only performed on a single patient (Patient 4). Furthermore, the postoperative follow-up for this specific patient was only 6 months. Concluding that this approach "demonstrates safety and feasibility" based on an  $n=1$  cohort with short-term follow-up is a significant scientific overreach. The authors must heavily revise the abstract and conclusion to reflect that this is a preliminary technical report or proof-of-concept, rather than a clinically validated standard.  
**Study Design Misclassification:** The manuscript is classified as a "Quality Improvement Study". It does not meet the criteria for quality improvement (e.g., PDSA cycles, institutional workflow changes). This is fundamentally a Retrospective Case Series detailing the medical records, diagnostic approaches, and surgical outcomes of four patients. The manuscript type must be re-categorized to align with its actual methodology.  
**Heterogeneous Outcomes:** The title implies a focus on a successful novel splenopexy, but the actual results show high morbidity and varied approaches among the four cases. Two patients required open total splenectomies due to splenic infarction. A third patient underwent a partial splenectomy with splenopexy, which failed within a month and required a secondary total splenectomy. Only the fourth patient had a successful splenopexy. The discussion should more critically analyze why the first splenopexy attempt in Patient 3 failed so rapidly compared to the reinforced technique in Patient 4.  
**Minor Revisions & Formatting Issues**  
**Redundancy:** The manuscript file currently contains duplicate sections, with the abstract and keywords appearing twice before the

introduction. The formatting needs to be cleaned up to adhere to standard journal submission guidelines. Image Annotations: The authors reference numerous CT scans and intraoperative photos (Figures 1 through 10). They should ensure that all graphical elements, particularly the schematic of the novel technique (Fig. 10), are rendered in high resolution with clear, standardized legends. Language and Tone: The English phrasing is generally proficient, but words like "remarkably brief interval" should be replaced with more objective scientific phrasing (e.g., "a 1.5-hour interval"). The foundation of the paper is solid—pediatric wandering spleen is exceptionally rare, and documenting these four cases contributes meaningful data to the literature. However, the authors must temper their claims regarding the novel splenopexy technique and accurately label the study as a retrospective case series

Major revision

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